

Differential Diagnosis

Though the clinical diagnosis is usually straightforward, other conditions that typically need to be considered are:

- **Tinea capitis:** Obvious inflammation (e.g., scaling, pustules) in association with patchy hair loss suggests a diagnosis of tinea capitis and excludes alopecia areata.
- **Trichotillomania:** This can be difficult to distinguish from alopecia areata (and may, in fact, be associated with it, especially in children). However, in trichotillomania, hair loss is rarely complete, and the broken hairs are usually firmly anchored in the scalp, unlike the easily pluckable exclamation point hairs seen in alopecia areata. A biopsy may be needed for clarification.
- **Early scarring alopecia:** May mimic alopecia areata clinically but lacks regrowth due to permanent follicular destruction.
- **Syphilis (alopecia areolaris):** Can present with patchy or diffuse hair loss; syphilis should be considered in the differential, especially with atypical features.

Diffuse alopecia areata can be difficult to differentiate from other causes of diffuse hair loss and may even be associated with them (e.g., thyroid abnormalities, other autoimmune diseases, and syphilis). Diffuse alopecia areata usually progresses rapidly, and the diagnosis often becomes clear over time, though a biopsy may be necessary to resolve diagnostic uncertainty.

Prognosis

Spontaneous remission can be expected in the majority of cases where hair loss is limited to a few small patches (possibly up to approximately 80 percent within 1 year), although most patients experience recurrences at some stage. The prognosis in extensive forms of alopecia areata—particularly **alopecia totalis** and **universalis**—is less favorable, with fewer than 10 percent of

patients achieving spontaneous recovery. The **ophiasis pattern** (band-like hair loss along the sides and back of the scalp) also tends to be recalcitrant.

Other features associated with a poor prognosis include: - Childhood onset

- Loss of body hair

- Nail involvement

- Atopy

- Positive family history for alopecia areata

Management

Counseling patients about the nature of alopecia areata, its variable prognosis, and available treatment options is essential. For most patients, alopecia areata is primarily a cosmetic concern. However, it can occasionally cause physical disability—such as when eyelash loss leads to ocular irritation or when significant nail dystrophy is present.

Despite its non-life-threatening nature, the cosmetic and psychosocial impact of hair loss can be profound, particularly in children and young women, though emotional distress is not limited to these groups. Given the limited efficacy of current treatments, the physician plays a crucial role in helping patients adapt to hair loss. This process may require support from other healthcare professionals, including clinical psychologists. Many patients benefit from involvement in patient support groups.

In pediatric cases, it is often the parents' emotional response that needs to be addressed to help the child adjust. The **National Alopecia Areata Foundation** (<http://www.naaf.org>) is an excellent resource for patient education and support.

Treatment

There is relatively little high-quality evidence from well-designed clinical trials; therefore, treatment recommendations are largely based on case series and clinical experience (Box 86-1). Helpful guidelines for managing alopecia areata have been published.

Box 86-1: Treatment of Alopecia Areata

Patchy Alopecia Areata

- **Intralesional corticosteroids:** Up to $2\,\mathrm{mL}$ per session, injected into the affected areas, repeated every 4–6 weeks.
- **Potent topical corticosteroids:** Applied 1–2 times daily.
- **Topical anthralin:** 0.1% to 2.0%, applied once daily.